UNITED STATES DISTRICT COURT
FOR THE EASTERN DISTRICT OF TEXAS
TEXARKANA DIVISION

Health Care Service Corporation, a Mutual Legal Reserve Company,
Plaintiff,
v.
Neuromonitoring Associates, LLC, Physician Oversight, LLC, and Monitoring Associates, LLC,
Defendants.

Case No.: 5:26-cv-00022-RWS-JBB

ORAL ARGUMENT REQUESTED

DEFENDANTS' MOTION TO DISMISS THE AMENDED COMPLAINT AND REQUEST FOR JUDICIAL NOTICE

Defendants Neuromonitoring Associates, LLC ("NMA"); Monitoring Associates, LLC; and Physician Oversight, LLC (together, "Defendants") move to dismiss Plaintiff Health Care Service Corporation ("HCSC")'s amended complaint (Dkt. No. 40) ("AC") under Federal Rules of Civil Procedure 12(b)(1) and 12(b)(6). The Defendants also request that the Court take judicial notice of the undisputed contents of the government forms and documents attached as Ex. 1–5.

I. INTRODUCTION AND SUMMARY OF THE ARGUMENT

In this case, HCSC offers three fraud theories, but each fails at the pleading stage. Bundling three deficient theories together in one complaint does not bolster any of them. This case should be dismissed with prejudice under Rules 12(b)(1) and 12(b)(6).

The first theory (the "NSA Theory") is foreclosed by Judge Schroeder's ruling in HaloMD. *Blue Cross Blue Shield of Texas v. HaloMD, LLC*, No. 5:25-CV-132-RWS, 2026 WL 1557492 (E.D. Tex. May 22, 2026) ("HaloMD"). This theory invokes RICO and state-law claims to seek review of No Surprises Act ("NSA") arbitration awards. But this is precisely the theory that the Court rejected in HaloMD. In light of that ruling, HCSC concedes it pleads these claims only to preserve them for appellate review. *See* AC at 38 n.24.

Indeed, in HaloMD, the Court held that the "NSA forecloses judicial review." 2026 WL 1557492, at *3. And HaloMD is indistinguishable: in both cases, the same affiliated plaintiff entities advanced the same theory through the same counsel, and both cases fail for the same reasons. In holding that the NSA bars judicial review, the Court reasoned that "inherent in the NSA's bar of judicial review of payment determinations is a limitation on the review of eligibility decisions" and that Blue Cross's claims were "an impermissible collateral attack on the [independent dispute resolution ("IDR")] awards." *Id.* at *4. The Court's decision was unequivocal and dismissed the case with prejudice for lack of subject-matter jurisdiction. Absent an intervening decision from the Fifth Circuit, the same outcome should follow here.[1]

The only addition to the NSA Theory here is a count seeking vacatur under the Federal Arbitration Act ("FAA"), 9 U.S.C. § 10. *See* AC at Count XI, p. 72 ("VACATUR OF NSA AWARDS UNDER 9 U.S.C. § 10"). HCSC asks the Court to vacate "thousands" (AC ¶ 220) of arbitration awards. It does not list or identify the thousands of arbitrations in question, state who the parties were, plead when they happened, or offer the required facts and evidence to show why they must be vacated. This fails both procedurally and substantively. The statute that HCSC

[1]Indeed, while HCSC and its affiliates have been filing these cases across the country, each court to rule on the theory to date has rejected it. *See Anthem Blue Cross Life & Health Ins. Co. v. HaloMD LLC*, 2026 WL 982629, at *7–10 (C.D. Cal. Apr. 9, 2026) (holding that Blue Cross plaintiffs failed to satisfy vacatur under the FAA where, as here, they "objected to eligibility for all the sample determinations identified in the" complaint but lost on this issue at arbitration, essentially "plead[ing] itself out of court"); *Aetna Health Inc. v. Radiology Partners, Inc.*, 2026 WL 1556164, at *3–4 (M.D. Fla. Apr. 16, 2026) (rejecting an insurance company's request to relitigate IDR arbitration awards that it called "fraud" based on arguments that it raised or could have raised during the arbitration and rejecting the insurance company's effort to use other statutes and causes of action "to end-around the NSA and FAA strictures," and held that use of state law to do so is preempted); *United HealthCare of Pa., Inc. v. Northstar Anesthesia of Pa., LLC*, 2026 WL 1145885, at *12–14 (E.D. Pa. Apr. 28, 2026) (holding that the court lacked jurisdiction over the insurer's common-law fraud claim where it sought equitable relief, recognizing that the insurer made "essentially the same arguments" as "rejected in Anthem," *supra*, and reasoning that the insurer was "trying to evade Congress's policy choices in limiting judicial review because UnitedHealthcare believes the [NSA] leaves it with an inadequate remedy"); Order at 7–11, *Guardian Flight LLC v. Aetna Life Ins. Co.*, No. 3:24-cv-00680 (D. Conn. June 16, 2026), Dkt. 389 ("Guardian Flight III") (granting motion to dismiss claims premised on eligibility for IDR because the NSA provides for an "exclusive procedure" to challenge awards and allowing the claims would "permit a workaround to the barrier to plenary judicial review Congress imposed").

invokes (9 U.S.C. § 10) expressly requires an "application" (not merely a complaint) filed by a "party to the arbitration" (which HCSC is not) and it must be supported by "clear and convincing evidence" (of which HCSC offers none). 9 U.S.C. § 10; *Morgan Keegan & Co. v. Garrett*, 495 F. App'x 443, 447 (5th Cir. 2012); *Garber v. Sir Speedy, Inc.*, 1996 WL 734947, at *4–5 (N.D. Tex. Dec. 11, 1996) ("The requirement to present such applications by motion rather than in the form of a complaint is founded upon the fundamental policy of the FAA to provide for expedited judicial review by preventing the losing party from relitigating the matter by means of filing a new suit in federal court." (citations omitted)). This procedural requirement has an important policy rationale: a party cannot file a notice-pleading complaint and then use discovery to investigate thousands of unidentified arbitrations in hopes of gathering the evidence needed to make its "application" later. *See Baylor Health Care Sys. v. Equitable Plan Servs., Inc.*, 955 F. Supp. 2d 678, 688 n.1 (N.D. Tex. 2013); *Garber*, 1996 WL 734947, at *4–5. And the FAA requires that such applications be filed within three months of the arbitration, which HCSC fails to do here. 9 U.S.C. § 12. Put simply, seeking blanket and summary vacatur of thousands of unidentified arbitrations flunks every requirement of the statute that Count XI invokes.

Nor does HCSC meet the substantive test for showing fraud that could not have been raised during the arbitration, or that any particular arbitrator exceeded its powers. *See* 9 U.S.C. § 10(a). In fact, not only does HCSC fail to plead facts to meet any of these four scenarios for review under the FAA, it includes allegations that categorically rule them out. It is a brightline rule under the NSA and FAA that the losing side in an arbitration cannot relitigate the matter in court simply by alleging that the other side's arguments were fraudulent, particularly where the losing side knew about and raised the same objections in the arbitration. *See Guardian Flight, L.L.C. v. Med. Evaluators of Tex. ASO, L.L.C.*, 140 F.4th 613, 620–21 (5th Cir. 2025) ("Guardian

Flight II"); *Barahona v. Dillard's, Inc.*, 376 F. App'x 395, 397–98 (5th Cir. 2010); *Anthem*, 2026 WL 982629, at *8. Likewise, that the arbitrator reached a decision that HCSC disagrees with does not mean that the arbitrator exceeded its powers; the losing side in any arbitration could try that argument, which is why precedent squarely rejects it.

* * *

HCSC's second theory (the "Practice-Acquisition Theory") relies on rhetoric to disguise why it fails as a matter of law. The following is HCSC's theory: years prior, NMA's competitors had arrangements to provide intraoperative neuromonitoring ("IOM") services where the surgeons owned the IOM companies to which they referred work. AC ¶¶ 42–43. A government agency opined that the competitors' approach could violate federal law as to claims submitted to the federal government. AC ¶¶ 44–46. After that, some surgeons sold their IOM companies to the Defendants, AC ¶¶ 49, and some did not switch to new providers, AC ¶¶ 78, 91, 104, 117. HCSC labels these corporate acquisitions as "kickbacks" and says they violate Texas law. AC ¶¶ 54, 65.

To be clear, HCSC does not allege a single instance where it claims IOM was not medically necessary, where HCSC was charged at a different price, or any other way in which the practice acquisitions affected it or harmed it, or that HCSC would be situated differently if those acquisitions had not happened or the surgeons' or the Defendants' businesses were structured differently. Instead, HCSC argues that the structure of the practice acquisitions violates Texas law. But HCSC does not have standing to litigate this: it has not suffered a concrete injury that is traceable to the challenged conduct. *See Lujan v. Defs. of Wildlife*, 504 U.S. 555, 560 (1992); *Earl v. Boeing Co.*, 53 F.4th 897, 903 (5th Cir. 2022) (no injury and no standing where passengers alleged they would not have bought tickets on an airplane that did not satisfy regulatory standards).

And there is another problem: the Texas laws that HCSC cites do not provide a private right of action. HCSC cannot sue under them directly, and has not attempted to do so here. Instead, it relies on a multi-step fraud theory: "The submission of a claim for reimbursement to HCSC also constitutes a certification and representation of compliance" with certain HCSC "Kickback Policies" which in turn supposedly require compliance with Texas law. AC ¶ 224 (emphasis added). The word "constitutes" does too much work in this sentence. Fraud is a lie, and there are no facts alleged to show a lie here. HCSC has not pleaded what the Defendants actually said that was false. It references "representations made on a claim form," AC ¶ 32, but never says what they are, notwithstanding its obligation under Rule 9(b) to plead with specificity. As HCSC notes, NMA submits claims using "standard healthcare 'claims forms'" (an example of this government form is attached as Exhibit 4). AC ¶¶ 29–32. For example, these forms make no representation about NMA's corporate structure or compliance with HCSC's policies. In short, its fraud count fails for lack of fraud.

* * *

Last, HCSC adds a third theory (the "Credentialing Theory") that amounts to no more than a vague rumor. According to an unnamed source, "some" IOM services were performed by providers who were either not licensed in the right state or not credentialed at the right hospital. It says that the Defendants use a "primary" and "secondary" reader for each IOM case. AC ¶ 124. While HCSC does not dispute that the primary reader was always licensed and credentialed, it says that sometimes the secondary reader was not credentialed or licensed in the relevant hospital or geography. *Id.* But only one reader is required, and HCSC has not pleaded a single case where either the secondary reader was not licensed or credentialed, or the primary reader did not in fact perform the reading. HCSC lacks standing here too. And it has pleaded no facts or

details whatsoever, despite the requirements of Rule 9(b). This claim amounts to nothing more than a rumor based on anonymous sources. *See* AC ¶ 128. This unsupported addition, haphazardly grafted onto the amended complaint, does not even satisfy the lower bar of Rule 8.

II. BACKGROUND AND PROCEDURAL HISTORY

A. NMA and IOM services

IOM services are procedures that monitor neural pathway integrity (i.e., nerve and brain function) in real time during certain neurosurgical, orthopedic, and vascular surgeries. AC ¶ 33. This involves off-site neurologists monitoring nerve function during surgeries. AC ¶ 36. There is no allegation or dispute about the medical necessity of these services, or that they were performed safely to proper standards.

NMA provides IOM services to independent surgeons nationwide. AC ¶ 39. To that end, NMA operates through several entities that furnish IOM services to doctors and file claims for reimbursement from insurance companies for their work. AC ¶¶ 14–16. Defendants Physician Oversight, LLC and Monitoring Associates, LLC are two such entities. AC ¶¶ 15–16.

Rather than using an outside company like NMA, some surgeons refer IOM work to companies that they own. But in 2023, the federal Department of Health and Human Services ("HHS") Office of the Inspector General ("OIG") issued Advisory Opinion No. 23-05, finding that self-referral to surgeon-owned IOM providers might violate the federal anti-kickback statute (which applies to claims for reimbursement submitted to the federal government, 42 U.S.C. § 1320a-7b) because the doctors own the companies to which they refer work. AC ¶¶ 42–44. After

[2]HCSC asserts that NMA requested that OIG issue the opinion. AC ¶ 47. This assertion is both false (and indeed, is pleaded without a good-faith basis under Rule 11(a)) and irrelevant to this motion.

the OIG opinion,[2] NMA expanded its business by acquiring IOM companies that were previously owned by surgeons, so that they could be operated in compliance with the opinion. AC ¶¶ 47–49.

B. The NSA and IDR process

Congress enacted the NSA to address surprise "balance bills" for out-of-network medical services. AC ¶¶ 137–138. The NSA included IDR, an arbitration process for "resolving payment disputes on claims between out-of-network providers and health plans." AC ¶ 140. There are three primary steps in IDR: open negotiations between the provider and the health insurer, IDR submissions, and then a binding payment determination by arbitrators known as Independent Dispute Resolution Entities ("IDREs"). 42 U.S.C. § 300gg-111(c); AC ¶ 142.

When a dispute arises over the payment amount for an out-of-network service covered by the NSA, either the medical provider or insurer initiates negotiations with the other side. 42 U.S.C. § 300gg-111(c)(1)(A); AC ¶ 141.d n.12. If they cannot agree on a payment amount within thirty days, either party can then initiate the IDR arbitration process. *See* 42 U.S.C. § 300gg-111(c)(1)(B); 45 C.F.R. § 149.510(b)(2)(i). A provider or insurance company initiates the IDR process through an online portal. AC ¶ 147. After a party initiates IDR, the parties select an IDRE (i.e., an arbitrator). AC ¶ 142.d. If they cannot agree on an IDRE, one will be appointed by HHS, a government agency. *Id.*; *see also* 42 U.S.C. § 300gg-111(c)(4)(F).

The amended complaint acknowledges that the first step of the IDR process is for the arbitrator to determine whether the claim is eligible for IDR. AC ¶¶ 142.f, 142.g, 142.h. The arbitrators are expressly authorized and indeed required to make this determination. AC ¶¶ 142.f, 142.g; *see also* 45 C.F.R. § 149.510(c)(1)(v). If the non-initiating party disagrees that a dispute is eligible, it can submit an objection to HHS, the Department of Labor, the Department of the Treasury, and the IDRE. *Anthem*, 2026 WL 982629, at *7. The IDRE reviews both the initiating

party's notification of IDR initiation and the non-initiating party's objection before deciding eligibility. Ex. 1, Federal Independent Dispute Resolution (IDR) Process Guidance for Disputing Parties, CMS.gov § 5.5 (2023); *see also* Ex. 2, Federal Independent Dispute Resolution (IDR) Process Guidance for Certified IDR Entities, CMS.gov 10–11 (2022). A dispute only moves forward—and there can only be an award—if the IDRE determines that it is eligible.

If the IDRE determines that the dispute is eligible, each party proposes a payment amount. AC ¶ 142.h. The IDRE then selects an offer it finds to be most reasonable, which is the amount the insurer must pay, based on criteria laid out in the NSA. *See* 42 U.S.C. § 300gg-111(c)(5)(B), (c)(5)(C). This is "baseball" style arbitration, where the IDRE must select between the two sides' offers and pick the one that is most reasonable under specified statutory factors. *See* H. Rep. 116-615, at 56–57 (2020). The IDRE may not consider the medical provider's usual or customary rates in determining the most reasonable offer. 42 U.S.C. § 300gg-111(c)(5)(D). Instead, the only factors that an IDRE may consider are the qualifying payment amounts ("QPAs"), information requested by the IDRE relating to the offers made, information provided by the parties relating to the offers, the provider's background and market share, the patient's acuity, the scope of services offered by the provider, and demonstrations of good faith efforts between the provider and insurer to enter into network agreements. *Id.* § 300gg-111(c)(5)(C).

The remedy for violating IDR rules is government enforcement, not private RICO litigation. As the Fifth Circuit has held, "HHS has the authority to enforce provider and payor non-compliance with the NSA's provisions." *Guardian Flight, L.L.C. v. Health Care Serv. Corp.*, 140 F.4th 271, 274 (5th Cir. 2025) ("Guardian Flight I") (citing 42 U.S.C. § 300gg-22(b)(2)(A) and stating that the statute "provid[es] for HHS enforcement against some payors for NSA non-

compliance," and citing 42 U.S.C. § 300gg-134(b) and stating that the statute "provid[es] for HHS enforcement against providers for NSA non-compliance").

C. HCSC's efforts to use litigation to re-write the NSA

HCSC is a large mutual legal reserve company that provides health insurance plans and administrative services for groups such as employers and governmental entities in five states. AC ¶ 13. Unhappy with its results during the IDR process, HCSC and its related Blue Cross entities have begun to flood the courts with collateral attacks on these rulings in federal district courts. This present case is part of HCSC's litigation campaign.

Here, HCSC filed its original complaint on February 19, 2026, and the Defendants moved to dismiss on April 21, 2026. As the motion to dismiss was pending, the Defendants moved to stay discovery given the weakness of HCSC's claims and the strength of the dispositive motion. Dkt. 32. This Court also decided HaloMD during this period. Two days before it was set to respond to the Defendants' motion to dismiss, HCSC filed the amended complaint rather than respond in opposition to the motion to dismiss. Dkt. 45, at 2. The next day, the Court granted the motion to stay discovery and held that the original motion to dismiss was moot. *Id.* at 5; Dkt. 42.

III. LEGAL STANDARD

The Defendants move to dismiss the amended complaint under Federal Rules of Civil Procedure 12(b)(1) and 12(b)(6). Under Rule 12(b)(1) a court must dismiss a case where it lacks subject-matter jurisdiction. "It is to be presumed that a cause lies outside [a federal court's] limited jurisdiction, and the burden of establishing the contrary rests upon the party asserting jurisdiction." *Pershing, L.L.C. v. Kiebach*, 819 F.3d 179, 181 (5th Cir. 2016) (alteration in original) (quoting *Kokkonen v. Guardian Life Ins. Co. of Am.*, 511 U.S. 375, 377 (1994)). "There

are two types of attacks against a court's subject-matter jurisdiction: 'facial' and 'factual.'" *Reardon v. Am. Airlines, Inc.*, 167 F.4th 294, 299 (5th Cir. 2026) (citation omitted). Here, the absence of subject-matter jurisdiction is apparent on the face of the amended complaint. *See id.*

Under Rule 12(b)(6), a court must dismiss a complaint that "fails to state a claim upon which relief can be granted." *Polnac v. City of Sulphur Springs*, 555 F. Supp. 3d 309, 321 (E.D. Tex. 2021). A complaint must be "plausible on its face." *Phillips v. Collin Cmty. Coll. Dist.*, 630 F. Supp. 3d 828, 833 (E.D. Tex. 2022). "A claim has facial plausibility when the plaintiff pleads factual content that allows the Court to draw the reasonable inference that the defendant is liable for the misconduct alleged." *Id.* (citation omitted); *see also Ashcroft v. Iqbal*, 556 U.S. 662, 678 (2009). Though courts take the facts alleged in the light most favorable to the plaintiff, legal conclusions and recitations of claim elements are not enough. *Iqbal*, 556 U.S. at 678.

The plausibility standard demands "more than a sheer possibility that a defendant has acted unlawfully." *Peacock v. AARP, Inc.*, 181 F. Supp. 3d 430, 433 (S.D. Tex. 2016) (quoting *Iqbal*, 556 U.S. at 678). "[F]actual allegations that are 'merely consistent with a defendant's liability, stop short of the line between possibility and plausibility of entitlement to relief,' and are thus inadequate." *Walker v. Beaumont Indep. Sch. Dist.*, 938 F.3d 724, 735 (5th Cir. 2019) (citation omitted). Rather, the plausibility standard "requires" that the plaintiff plead facts demonstrating "that there is no 'obvious alternative explanation' for the decision" that is lawful. *Pickett v. Tex. Tech Univ. Health Scis. Ctr.*, 37 F.4th 1013, 1034 (5th Cir. 2022) (quoting *Iqbal*, 556 U.S. at 682); *see also Guardian Flight II*, 140 F.4th at 622 ("A claim is merely conceivable and not plausible if the facts pleaded are consistent with both the claimed misconduct and a legal and obvious alternative explanation." (citation omitted)); *Bell Atl. Corp. v. Twombly*, 550 U.S. 544, 567 (2007).

Respectfully submitted,

McDermott Will & Schulte LLP

/s/ Matthew L. Knowles
Matthew L. Knowles
(pro hac vice) (Lead attorney)
200 Clarendon Street
Boston, MA 02116
(617) 535-4000 (telephone)
mknowles@mcdermottlaw.com

Attorneys for Defendants Neuromonitoring Associates, LLC, Physician Oversight, LLC, and Monitoring Associates, LLC

CERTIFICATE OF SERVICE

I hereby certify that on June 18, 2026, the foregoing document was served on all counsel of record through the Court's CM/ECF system, which sends notice of the filing by e-mail to all counsel of record.